

Palliative Care & Symptom Management in Oncology

Pain Control, Cachexia, Psychological Support, and End-of-Life Planning

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1. Integrating Palliative Care

The ASCO and ESMO guidelines recommend early integration of palliative care alongside oncological treatment for all patients with advanced cancer. The landmark Temel et al. (NEJM 2010) trial demonstrated that early palliative care in metastatic NSCLC improved quality of life, reduced aggressive end-of-life interventions, and unexpectedly improved median OS by 2.7 months compared with standard care alone.

Palliative care is NOT synonymous with end-of-life care. It is a parallel supportive strategy applicable at any stage of cancer, from diagnosis through survivorship and into end-of-life.

2. WHO Cancer Pain Ladder

Step	Pain Severity	First-Line Agents	Adjuvants
Step 1	Mild (NRS 1-3)	Non-opioid: Paracetamol 1g QDS ± NSAID	Gabapentinoids, TCAs, SNRIs for neuropathic component
Step 2	Moderate (NRS 4-6)	Weak opioid: Codeine, Tramadol, low-dose morphine/oxycodone	Continue adjuvants; laxatives mandatory
Step 3	Severe (NRS 7-10)	Strong opioid: Morphine, Oxycodone, Hydromorphone, Fentanyl	Dexamethasone (bone pain); ketamine (refractory); methadone (specialist)

3. Opioid Prescribing Principles

- Start with oral morphine immediate-release (IR) q4h + breakthrough dose (1/6th of 24-hour total dose q1h PRN)
- Convert to modified-release (MR) once stable: total 24h IR dose $\div$ 2 = twice-daily MR dose
- Opioid rotation: Switch when inadequate analgesia or intolerable side-effects; reduce equianalgesic dose by 25-30% for incomplete cross-tolerance
- Equianalgesic conversions (approximate): Oral morphine 30 mg = oral oxycodone 20 mg = oral hydromorphone 6 mg = transdermal fentanyl 12 mcg/hr
- Always prescribe prophylactic laxatives with opioids (macrogol + senna); antiemetics for first 1-2 weeks
- Anticipate respiratory depression: Naloxone 400 mcg IV/SC for RR <8/min or SpO2 <90% with unresponsiveness

4. Management of Common Oncology Symptoms

Symptom	First-Line Management	Second-Line / Specialist Options
Nausea/Vomiting	Ondansetron $\pm$ dexamethasone; metoclopramide for gastric stasis	Haloperidol, levomepromazine; lorazepam (anticipatory N&V;)
Dyspnoea	Low-dose oral/SC morphine (evidence-based); fan/cool air; oxygen if hypoxic	Midazolam SC in terminal phase; ventilatory support (non-invasive)
Delirium	Identify and treat reversible causes (hypercalcaemia, infection, opioid toxicity); haloperidol 0.5-2mg	Quetiapine (hypoactive delirium); midazolam if agitated and refractory
Constipation	Macrogol + stimulant laxative (bisacodyl/senna); adequate hydration	Methylnaltrexone (PAMORA) for opioid-induced; manual evacuation if impaction
Fatigue	Treat reversible causes (anaemia, hypothyroidism, depression); corticosteroids short-term	Exercise programme; methylphenidate (limited evidence); sleep hygiene

5. End-of-Life Care Planning

- Advance Care Planning (ACP): Discuss goals of care, preferred place of death, CPR and DNAR decisions, ADRT documentation
- Recognising dying: DNAR already in place, no further anticancer treatment, deteriorating despite optimal medical management, patient/family aware
- Liverpool Care Pathway successor: Individualised care plans (ICP); review medications (discontinue non-essential drugs), maintain comfort
- Continuous deep sedation (palliative sedation): For refractory suffering in the imminently dying; midazolam continuous SC infusion; requires full informed consent and documentation
- Bereavement support: Routine follow-up contact with family; refer to specialist bereavement services for complicated grief

■ **HYPERCALCAEMIA OF MALIGNANCY:** Corrected Ca >3.0 mmol/L (12 mg/dL) is a medical emergency causing delirium, renal failure, and cardiac arrhythmia. Treat with IV 0.9% NaCl rehydration + IV zoledronic acid 4 mg over 15 min. Denosumab for bisphosphonate-refractory cases.